Case study: Hannah

Condition: **Stroke – left posterior cerebral artery (occipital) stroke**

29-year-old woman · Community rehabilitation service, outpatient appointment

Hannah Kowalski is a 29-year-old woman who had a **left posterior cerebral artery (occipital) stroke** eight weeks ago, two weeks after giving birth to her daughter, Mia. Hannah was discharged home from hospital after a short admission and now attends a community rehabilitation service. She is a primary school teacher on parental leave. Hannah lives with her partner, Ryan, in a two-storey townhouse, but Ryan works fly-in fly-out and is currently away for two weeks.

Hannah's discharge summary notes that she walks independently, and she has had no falls since discharge. At her first community appointment last week, she scored 28/30 on a cognitive screen. Hannah tells you that her GP saw her recently and "said I'm fine".

You have been asked to complete an occupational therapy session focused on Hannah's goal of returning to teaching next year. Hannah arrives on time, carrying Mia, now 10 weeks old, in a baby capsule. She is articulate and confident and has brought a typed draft of a return-to-work plan. She asks for your help writing a letter to her school principal. "I'm basically back to normal," she says. "I just need a plan."

As you talk, Hannah mentions several recent events, laughing them off. She has bruised her right hip twice on the corner of the kitchen bench and says she has become "so clumsy since the baby". Last week, in a shopping centre car park, a car reversing on her right "came out of nowhere", and she had to pull the pram back quickly after a passer-by shouted a warning. She carries Mia up and down the stairs several times a day and has caught the capsule on the right-hand stair rail more than once. When bathing Mia on the bathroom bench, she twice did not notice that the hot tap on her right was still running until the water felt too warm.

You ask Hannah to read the first paragraph of her plan aloud. She leaves out words at the right-hand end of several lines, then stops and says the sentences "don't make sense", before rereading slowly and moving her head. When you slide a second page to the right side of the desk, she does not reach for it until you point it out. When a colleague walks into the room on her right, Hannah startles.

Hannah's mother lives in New Zealand and has offered to fly over and stay, but Hannah says, "I don't want to make a fuss, I'm managing." There is no clear documentation of whether Hannah's vision has been formally assessed since her stroke, whether she has been given advice about driving, or what support she has at home while Ryan is away. It is also unclear what the GP reviewed at her recent appointment.

Towards the end of the session, Mia starts crying loudly and cannot be settled. Hannah apologises, says Mia needs a feed, and starts packing up. When you ask how she got here, she says she drove, as she has done for the past few weeks: "I just turn my head a bit more." She picks up her car keys and Mia's capsule and says she will stop at the shops for nappies on the way home. "Can we do the letter for my principal next week?"

You are the occupational therapy student completing Hannah's appointment and your clinical supervisor is not currently in the room.

Present Hannah's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

Unofficial practice material. All cases are fictional and written for ISBAR communication practice; they are not clinical guidance. Not produced by or affiliated with any university.